

Five priorities for qualified patient inquiries

LACATC, Encino, California | October 7, 2026 | elev8 assessment

Decision: start with payer clarity and callback friction. The 38-page public crawl found HTTP 200 responses, self-canonicals, call links and parseable JSON-LD throughout. Priorities below reflect proximity to admission, reach and confidence. Business impact is a hypothesis: GSC, call outcomes and admission data were unavailable. AI coding tool: OpenAI Codex, ChatGPT Work Mode.

1. Payer promises vary across the same landing page. The [Medi-Cal page](#) qualifies coverage in its introduction, while its search description and related detox card imply broader coverage. This can misqualify inquiries or weaken confidence among suitable patients. **Fix:** have admissions validate program/plan facts, then align metadata, cards, body and relevant schema. **First check:** GSC payer queries/landing pages; call-to-admission rate and payer-related disqualifications. **Evidence E1; high observation confidence, medium impact confidence.**

2. Callback forms demand two contact channels. Both forms on [Admissions](#) require phone and email; source and rendered DOM agree. The shared pattern appears on 34 intent-matched crawl pages. Extra effort may lose visitors who prefer a callback; telephone links remain available. **Fix:** test required phone with optional email and preserve consent. **First check:** organic mobile landing traffic, field abandonment, connected callbacks and qualified admissions per visitor. **E2; high observation, medium impact confidence.**

3. Provider credentials require extra investigation. The [About page](#) directs visitors to request license details by phone; the reviewed page supplies neither a facility-specific verification link nor named clinical leadership. Families assessing trust face another step. **Fix:** publish verified operator/license details, regulator links and clinical biographies beside program claims. **First check:** branded trust queries and qualified callers' hesitation/lost-lead reasons. **E3; high observation, medium impact confidence.**

4. AI retrieval can encounter conflicting detox answers. The [detox program FAQ](#) gives a categorical reassurance about pain; the [detox guide](#) acknowledges discomfort and variation. Both answers surfaced in a search-retrieval probe. An AI summary could select the broader reassurance and set the wrong expectation. **Fix:** clinical review, consistent answers and a review date. **First check:** GSC detox queries/landing pages and qualified calls from identifiable AI referrals; separately repeat ChatGPT/AI Overviews/Perplexity prompts. Consumer-AI answer share remains unmeasured. **E4; high source confidence, low-medium impact confidence.**

5. Age eligibility is hard to establish before contact. Reviewed admissions, residential and family pages give no explicit minimum admission age. This leaves parents uncertain about fit. **Fix:** publish the provider-approved age range and referral route on those entry pages. **First check:** age-related GSC demand and age-ineligible call dispositions; validate frequency before a larger content investment. **E5; medium observation/impact confidence.**

Measurement: compare answered, qualified organic inquiries, assessments and admissions by landing-page cohort. Click-to-call events are only proxies. Public-only review; no forms submitted, calls placed or facility contact. Evidence, exclusions and limits: [evidence/findings.md](#).